جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عواطف فيصل تميم حسين
المحافظة : القاهره **الحي / المدينة :** اخري **العنوان :** -*-/-/-
رقم البطاقة : 26605032600589 **رقم التليفون :** 01153391421 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 03-05-1966 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
الإخصائي الاجتماعي
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عواطف فيصل تميم حسين
National ID: 26605032600589

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





DMS

نوصيا

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

لدعم صحة المرأة المصرية

مصر

MDT FORM 27/2/2026 G

Name	حواطف نبيل عبد	MRN	5825/24
		ID	
Age	59 y.o	Phone	
Menopausal status	Post.	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	HTN.		

Clinical Data

BX: IDC GII

ER:	PR:	HER 2:	Ki67:
8/8	8/8	+2 not done.	

Sonomammography: (MBC to bone, liver, lung) (2.22) → MBC to bone. → AI x 2 cy. (10-2-24) → D.P

Mets Workup: by Neuro develop WBRT. Fasloder, Ibrance x 4 cycl → (6-2-25) D.P. st alone, Arimidex. for (6-2-25)

R L

TNM

Recommendations: CT/S → S.D. PS: 3

Bone Scan (9-10-2-25) → D.P.

MRI brain (17-1-2-26) → Rt temporal deep white matter area Signals abnormality likely Mets.

pt declined Chemo → for assessment of PS first at Clinic

MDT OUTCOME & PLAN Recommend: - New brain MRI + C - Start Xeloda single agent.

for MRI brain + PS → start Xeloda Single agent

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

[[DOCTYPE:MDT_FORM]]

MDT form

A-Patient ID:
1- Name : عوطاف أيمن كمب حسن Sex: Female 3- Medical number: 83241157 Old Med: 3825-2024

2-ID number: 26605032600589 Patient Age : 59 Y

Family history	-ve	Phone	01229266065	BMI	REASON OF MDT
marital status		NRS Score		GROUP	Date of MDT
Co morbidity	-ve	Menopausal status		Initial STAGE	

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC gll	8/8	7/6	1		40

T	N	M
		Ibone LIVER

Sonomammography

PATIENT HISTORY

MBC to bones since 10/2022-->femara x 2 yrs
14/10/2024 BS-->PD of bone deposits
CT 4/11/2024 > DP & NEWLY DEVELOPED hepatic focal lesion & newly pulmonary nodules
Recieved 4 cycles Faslodex/Ibrance last 5/2025
Assessment with CT CAP showd PD on HFL bonyb mets and bil Pulmonary noduules along v

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for new biopsy to confirm her IHC on the current disease
shift to third lione Affinator/Aromazine + zometa q 3month

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY	SURGERY	PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / عواطف فيصل تميم حسين	Request No.	: 13008293
Patient No.	: 26605032600589	Request Date	: 28/07/2025
Path No.	: 2390/25	Verification Date	: 05/08/2025
Sex	: Female	Financial Category	: روسي - نفقة الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada H

Pathology Report

Clinical Data: Segment IVa hepatic focal lesion, in patient with CT showing regressive course regarding a previously noted left breast mass.
Nature of Specimen: Tru-cut biopsy, collected at 27/07/2025.

Gross Examination:
- Two grey white tissue cores, the longer was 1.5 cm long. Totally submitted.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic Examination:
Histopathological examination reveals cores of hepatic tissue showing infiltration by tumor formed of malignant epithelial cells showing moderate nuclear pleomorphis & moderate tendency to tubular formation, arranged in nests and trabeculae.

* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
(Average Her2 copy No.: (2.8), Her2/CEP17 ratio: (1.5)).
- Ki67%: 30%.

Diagnosis:

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadeema - Cairo, Egypt

Patient Name	:الأستاذه / عواطف فيصل تميم حسين	Request No.	:13008293
Patient No.	:26605032600589	Request Date	:28/07/2025
Path No.	:2390/25	Verification Date	:05/08/2025
Sex	:Female	Financial Category	:وسي - نقلة الدولة.
Age	:59 Y	Department	:Out Patient
Referreing Doc	:	Registered by	:Mrs.Mayada H

Pathology Report

Segment IVa hepatic focal lesion, Tru-cut biopsy:
- Findings are compatible with metastatic carcinoma, Likely of breast origin.

IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.

(Average Her2 copy No.: (2.8), Her2/CEP17 ratio: (1.5)).
- Ki67%: 30%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

Verified by :
Dr.Duaa Mohamed

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Patient Name	:الأستاذه / عواطف فيصل تميم حسين	Request No.	: 13008293
Patient No.	:26605032600589	Request Date	: 28/07/2025
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Sex	:Female	Financial Category	: وسي - نقة الدولة
Age	:59 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada H

Pathology Report

• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell i more than 4.
• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cel less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell i more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell more than 4 but less than 6. (Recounting give the same result).
• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell less than 4.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

PATHOLOGY REPORT

Name: عواطف فيصل تميم حسن
Patient's I.D :26605032600589
Age: 56 Y.

Pathology Number: 370/2022
Specimen Received Data: 24 /
Date Report Issued: 25/ 10 /202

Clinical data: Bilateral breast masses.

Nature of the specimen: U.S. Guided Tru Cut Biopsy.

GROSS EXAMINATION: Two specimens received, labeled as (1) right & (2) left; each formed of tissue cores, both were totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined from *specimen (1)* designate (right side) revealed mammary tissue showed lobular adenosis, fibrosis and cystically dil ducts; some showed intraluminal retained secretions and foamy histiocytes. *No evidenc malignancy*. Serial sections examined from *specimen (2)* designated as (Left side) reve cores of malignant tumor formed of cords and tubules of moderately anaplastic epith cells. Surrounding desmoplastic stroma was seen.

DIAGNOSIS: Bilateral Breast Masses, U.S. Guided Tru Cut Biopsy:

1. Right side: **Fibrocystic Change Of The Breast; No Malignancy.**
2. Left side: **Invasive Duct Carcinoma, NST, Grade II.**

PROF. DR. SOMIA ABDEL LATIF
DR. AYA MAGDY

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Presidential Initiative for Women's Health

Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

مستشفيات جامعة القاهرة

IMMUNOHISTOCHEMISTRY REPORT

Name: عواطف فيصل تميم حسن	TM: 370 /2022
PATIENT'S I.D :26605032600589	Specimen Received Data: 26/ 10 /2022
Age: 56 Y.	Date Report Issued: 30/ 10 /2022

CLINICAL DATA: A case of *left* breast cancer, for estrogen, progesterone receptors, Her-2/neu and Ki-67

PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu. (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2). CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana IView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

RESULTS:

Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.

Examination of section treated for progesterone receptors revealed strong positive nuclear staining in 40% of tumor cells by chromogen.

Examination of section treated for Her-2/neu revealed weak incomplete membrane staining in +10% of tumor cells by chromogen (score 1+).

Examination of section treated for Ki67 revealed positive nuclear stain in average of +40% of tumor cells.

CONCLUSION:

Tumor Cells Were:

- *Positive* For Estrogen Receptors, Score 8/8.
- *Positive* For Progesterone Receptors, Score 7/8.
- *Negative* For Her-2/neu (Score 1+).
- 40% Proliferative index

COMMENT: *All controls show appropriate reactivity.*

Prof. Dr. Ahmed Abdel Aziz

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Awatef Faisal Tamim Hussein	Date: 27/04/2025
Age: 59 Year(s)	Patient ID: 93870
Exam: CT CHEST WITH CONTRAST	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Compared to the previous study dated 04/11/2024 the current study shows:

I Chest:-

- Regressive course regarding the size of the previously noted left breast LOQ soft tissue mass lesion currently measures about 4.3x3 cm as compared to 4.7x3.5 cm.
- Mild regressive course regrading few pathologically enlarged left axillary lymph nodes with largest measuring 16x10 mm as compared to 18 x 10 mm at level-I.
- Stationary course regarding left supraclavicular subcentimetric lymph node measures about 11x8 mm.
- Mildly increased size regarding some of the previous multiple small pulmonary nodules however, the largest still measuring 8 x 6 mm along the horizontal fissure.
- Newly developed right lower lung posterior and medial segment soft tissue density measures about 4.2x2 cm.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size showing progressive size regarding the previous hypodense focal lesion at segment IVa, measuring 3.4x2.9 cm as compared to 2.6 x 2.8cm and newly developed another focal lesion seen at the right hepatic lobe segment VII measures about 1.5x1.2 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.

CT Scan
[[DOCTYPE:CT_SCAN]]

عواطف نبيل

c.c 9 / 8 / 26

جامعة الأزهر
مستشفى الحسين الجامعي
قسم علاج الأورام و الطب النووي

To whom Who may Concern,

♀ Pt 59 y. HTN

A Case of De-novo met breast to bone, liver & brain

Referred to us for BoBI

Pt Received WBI 30 Gy / 10 ##

Started on 10 / 8 / 2026

finished on 24 / 8 / 2026

Please, for Your Kind Care

د. ابراهيم ماجد
نائب مدير الإدارة

شارع جوهر القائد، الدراسة، القاهرة تليفون (+٢) ٢٥١١١٩٨١ - (+٢) ٢٥٠٦٣٥٦٠
Gawhar Al-kaed St, Al-Darrasa, Cairo Egypt phone& Fax (+2) 25111981 - (+2) 25063560

Patient's Name:	**Awtif Faisal Tameem Hussein**
Date:	**2026-08-29**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Post Contrast Multi-Slice CT scan of the Chest, Abdomen and pelvis Revealed

- ***Known case of Metastatic left breast cancer on follow up***

Chest findings

- ***Still noted bilateral multiple small pulmonary nodules the largest seen at the left upper lung lobe measuring about 10mm.***
- *Bilateral basal thick atelectatic bands.*
- *Patent trachea, main and segmental bronchi*
- *Normal both **hilar regions**. No hilar masses or lymphadenopathy.*
- *Clear **pleural surfaces** with no evidence of pleural effusion.*
- *Normal **cardiac** size, no pericardial effusion*

Abdomen and pelvis findings

- ***Stationary course** regarding the previously described multiple bi-lobar hepatic focal lesions the largest seen at segment IV and measures about (7.4x4.2cm).*
- *No dilated intra-hepatic **biliary radicles.***
- *Normal caliber patent **Portal Vein***
- *Normal **gallbladder**. No hyperdense gall stones. Normal extrahepatic bile ducts.*
- *Normal size, shape and parenchymal density of the **pancreas**. No enhancing lesions.*
- *Normal size, shape and density of the **spleen**. No focal lesions.*
- *Normal size, and contrast excretion by **both kidneys**. Normal parenchymal thickness. No renal, masses stones or back pressure changes.*
- *Normal both **supra-renal glands**, no masses.*
- *No **ascites.***
- *No significant abdominal or pelvic **lymphadenopathy***
- *Normal **aorta, I.V.C.***
- *Normal filling of the **urinary bladder** with smooth inner surface and uniform wall thickness.*
- *No **pelvic collections.***
- ***Bulky uterus with well defined left wall relatively heterogenous lesion measuring about ((4x 4.2 cm)...Likely submucous fibroid for US correlation.***
- ***Extensive sclerotic bony lesions seen involving the all the scanned bones.***

CONCLUSION:

- ***Near stationary course.***

Ibrahim

Ibrahim

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Date:	**2026-08-29**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

OPEN MR EXAMINATION OF THE BRAIN

TECHNIQUE:

Axial T1, T2, FLAIR, diffusion & ADC WIs
Coronal T2WIs
Sagittal T1WIs
Post contrast T1 WIs

FINDINGS:





- **Sill noted** right anterior temporal extra-axial meningeal thickening with mild meningeal enhancement is seen exerting mass effect in the form of effacement of the underlying cortical sulci at the right temporal region with high T2 and FLAIR signal.
- Mild chronic white matter ischemic changes with few lacunar infarctions seen scattered in both cerebral hemispheres.
- Age related brain involutional changes
- No midline shift or deformity.
- No evidence of cerebral, cerebellar or brain stem intra-axial enhancing mass lesions.
- No cerebello pontine angle masses.
- No evidence of posterior fossa abnormalities.
- Intact cervico medullary junction.

OPINION:

Stationary course.

عواطف نبيل

c.c 9 / 8 / 26

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مستشفى الحسين الجامعي
قسم علاج الأورام و الطب النووي

To whom Who may Concern,

♀ Pt 59 y. HTN

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Pt Received WBI 30 Gy / 10 ##

Started on 10 / 8 / 2026

finished on 24 / 8 / 2026

Please, for Your Kind Care

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نائب مدير الإدارة

شارع جوهر القائد، الدراسة، القاهرة تليفون (+٢) ٢٥١١١٩٨١ - (+٢) ٢٥٠٦٣٥٦٠
Gawhar Al-kaed St, Al-Darrasa, Cairo Egypt phone& Fax (+2) 25111981 - (+2) 25063560

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Abdomen and pelvis findings

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Ibrahim

Ibrahim

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Date:	**2026-08-29**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

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- No cerebello pontine angle masses.
- No evidence of posterior fossa abnormalities.
- Intact cervico medullary junction.

OPINION:

Stationary course.

Medical No. : 2671
Patient's Name: Awtif Faisal Tameem Hussein
Date: 2026-10-14
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with multiple bilobar hypoechoic focal lesions, the largest is seen at left lobe measures 7x6cm*
- no *intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with no two large stones 16& 17mm seen inside
- *. The wall is not thickened.*
- *CBD is not dilated.*

Spleen: -

- *Average size, homogenous.*

- **Kidneys:** -
  - *Both are average in size, shape and echogenicity*
  - *No stones or dilated pelvicalyceal system*
  - *No gross lesion*

Para-aortic and pancreatic area: *show no gross abnormalities.*

No ascites

Urinary Bladder: -
- not distended yet *no stones or diverticular out pouching*

Uterus and adnexal: *no abnormalities could be detected apart from anterior uterine wall hyperechoic lesion measures 3x2.5cm likely lipoliomyoma*

Opinion:

H***epatic FLs likely metastases***

Calcular GB

Uterine lipoliomyoma

Dr/Nermeen Ramadan.MD

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Awtif Faisal Tameem Hussein	Date : 07-09-2026 03:49
Branch : Dokki	Medical No. : 456573
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

NILE DIAGNOSTIC CENTER

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 60-year-old female with left breast carcinoma, status post-surgery, chemotherapy, radiotherapy, and current Zoledronic acid therapy. Referred for follow-up skeletal evaluation compared to prior study dated October 9, 2025.

Findings:

- Widespread multifocal areas of increased tracer uptake are noted throughout the axial and appendicular skeleton, including the calvarium, skull base, sternum, bilateral clavicles, scapulae, shoulder girdles, multiple bilateral ribs, entire vertebral column, pelvic girdle (iliac bones, sacrum, ischial and pubic rami), and proximal-to-mid shafts of both humeri and femora.
- Compared to the prior study (October 9, 2025), there is significant improvement with regression of the prior superscan pattern. Renal parenchymal uptake and urinary excretion are now clearly visualized with restored soft-tissue background clearance.
- Select lesions show mild reduction in tracer avidity.
- No new lesions or photopenic defects are identified.

Impression:

- Widespread skeletal metastatic disease showing interval regression / partial scintigraphic response compared to the prior study (October 9, 2025), evidenced by resolution of superscan appearance, recovery of renal excretion, and normalized background clearance.
- Clinical correlation and continued follow-up are advised.

Dr. Alaa Nagi MD Nuclear Medicine

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

10/11/2026

MDT FORM

Name:	Eawatif Faycal Tamim Huseen	**MRN:**	96
Age:	60 Y	**ID:**	26605032600589
Menopausal Sta	postm	**Phone:***	01220807046
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	dp visceral crisis
Surgical History:*	none	**ECOG Performance Status**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4

Clinical Data:

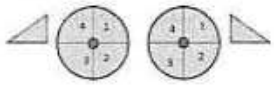
BX:

ER: **PR:** **HER2:** **KI67:**

History:

postm left breast cancer met to bone liver lung meninges
idc 2 er8/8 pr8/8 her22+ sish not amplified .
history; 2022 mets to bone ai x2 y till 10/2024
newly dev hfl rec fasoldex ibrance x4 cycles dp afinitor aromasin till 10/2025 dp of osseous lesions st xeloda 2/2026 mri brain rt temporal extraxial meningeal thickening with mass effrect . mri brain 5/2026 more enhanced lesion rt temporal meninges cts 24/5 rd of hfl pt has improved facial n palsy ptb rec wci 8/2026 mri brain 29/8 sd
cts 29/8 sd of bilobar hfls largest seg 4 m 7.4x4.2
bs mild rd but t bil elevated 2.35 (previous 1.1)
for mdt visceral crisis

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

FOR US ABD to rule out dilated CBD
TO start taxol /carbo weekly

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د حنان عزت	د احمد لجويب		